

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT INFORMATION****

Name: Patient 09

Age: 70 years old | Sex: Male

MRN: 000-TEST-09

Admission Date: [DATE] | Discharge Date: [DATE]

Facility: Metro Medical Center

****PRIMARY & SECONDARY DIAGNOSES****

- Congestive Heart Failure with reduced ejection fraction (EF 32%)
- Benign Prostatic Hyperplasia (BPH) with nocturia
- Gout (chronic, well-controlled)
- Hypertension
- Hyperlipidemia

****HOSPITAL COURSE****

Patient 09 presented with a 3-day history of progressive dyspnea on exertion (DOE), orthopnea, and lower extremity edema. Initial vitals showed BP 158/94, HR 98, SpO2 92% on room air. Chest X-ray confirmed pulmonary edema with bilateral infiltrates. BNP was elevated at 487 pg/mL (normal <100). Transthoracic echocardiogram revealed global left ventricular dysfunction with EF 32%, consistent with systolic heart failure.

Patient was admitted to telemetry. IV furosemide was initiated at 40 mg q.d. with diuretic titration based on daily weights and urine output. Over the 4-day inpatient stay, the patient

diuresed approximately 5.2 kg. Dyspnea improved significantly; SpO2 remained 94–97% on room air by discharge. Beta-blocker was optimized and ACE inhibitor initiated with tolerability monitored (baseline Cr 1.1, discharged at 1.3 — acceptable range).

No acute arrhythmias, troponin elevations, or acute coronary syndrome identified. Patient ambulated without difficulty by hospital day 3 and tolerated a cardiac diet without complications.

****DISCHARGE MEDICATIONS****

Medication	Dose	Frequency	Duration
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Lisinopril (ACE-I)	10 mg	q.d.	ongoing
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Metoprolol XL (beta-blocker)	50 mg	q.d.	ongoing
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Furosemide (loop diuretic)	40 mg	q.d.	ongoing; titrate PRN per weight
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Atorvastatin	40 mg	q.d. at bedtime	ongoing
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Allopurinol (gout prophylaxis)	100 mg	q.d.	ongoing
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Tamsulosin (BPH)	0.4 mg	q.d. at bedtime	ongoing
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Aspirin	81 mg	q.d.	ongoing
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****ACTIVITY & PHYSICAL RESTRICTIONS****

- Ambulation as tolerated; no strenuous exertion or heavy lifting >10 lbs
- Cardiac rehab referral provided; recommend enrollment within 2 weeks
- Return to normal ADLs gradually over 1–2 weeks

****DIETARY RECOMMENDATIONS****

- Strict fluid restriction: 1.5 L per day (including all beverages, soups, ice)
- Sodium restriction: <2 g/day
- Continue cardiac diet; avoid processed foods

****DAILY WEIGHT MONITORING****

- Weigh each morning after void, before breakfast
- Report weight gain >2–3 lbs in a single day or >5 lbs in one week to cardiology

****FOLLOW-UP APPOINTMENTS****

1. Cardiology clinic — in 1–2 weeks (call 000-0000 to schedule)
2. Primary Care Physician — in 1 week for medication reconciliation and BP check
- 3.